

Policy on Meeting Information Needs

«Hospital Name»

Document control

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Prepared by	IT/MIS In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

Statement of intent

«Hospital Name» identifies and meets the information needs of patients, visitors, staff, management and external agencies, so the right information reaches the right person at the right time.

1. Purpose

This policy sets out how «Hospital Name» identifies the information needs of patients, visitors, staff, management, external agencies and the community; captures and disseminates those needs; acquires information management and technology commensurate with them; maintains IT and communication network infrastructure; ensures information resources are accurate; contributes to external databases; and works toward digital health technology.

This policy does not cover data management processes, medical record content, or confidentiality and security of records — those are covered in other hospital policies.

2. Scope

This policy applies to the information needs of all stakeholders of «Hospital Name» — patients, visitors, staff, management, external agencies and the community.

3. Policy standards

«Hospital Name» identifies the information needs of patients, visitors, staff, management, external agencies and community through a systematic process.

«Hospital Name» captures and/or disseminates identified information needs.

«Hospital Name» acquires information management and technology commensurate with identified information needs.

«Hospital Name» implements a maintenance plan for information technology and the communication network.

«Hospital Name» has a contingency plan to ensure continuity of information capture, integration and dissemination.

«Hospital Name» ensures information resources are accurate and meet stakeholder requirements.

«Hospital Name» contributes to external databases in accordance with law and regulations.

«Hospital Name» makes efforts to use digital health technology to improve operational efficiency, patient safety and patient experience.

4. Non-negotiable rules

1. Do not operate without identifying, through a systematic process, the information needs of patients, visitors, staff, management, external agencies and the community.
2. Do not leave an identified information need uncaptured or undisseminated.
3. Do not acquire information management or technology solutions unrelated to identified current and future information needs, or without the necessary software licence.
4. Maintain an IT and communication network maintenance plan at all times, including fire protection for data servers and telephone exchange units.
5. Do not leave information capture, integration and dissemination without a contingency plan for system downtime.
6. Do not disseminate an information resource — website, brochure, newsletter, flyer, educational material — that has not been checked for accuracy.
7. Do not withhold legally required information from external agencies or miss a required submission — including notifiable disease reporting and national vigilance programmes.
8. Staff who see a rule under this policy broken report it the same shift to the IT/MIS In-Charge or the Medical Superintendent.

5. What we do

5.1 Identifying information needs

«Hospital Name» identifies the information needs of patients, visitors, staff, management, external agencies and the community through a systematic process — for example feedback forms, patient calls, focus group interviews or benchmarking. Identified information needs are documented. This covers, for example, OPD timings and service availability for patients; visiting hours and age restrictions for visitors; leave policy and standard operating procedures for staff; daily census and utilisation rates for management; vital statistics and notifiable-disease data for external agencies; and information on new services or new medical staff for the community.

5.2 Capturing and disseminating information needs

Written guidance at «Hospital Name» covers capture and/or dissemination of identified information needs, and is implemented. Dissemination may be through a website, intranet, information booklets, display or signage; external agencies are provided information per applicable methodologies and guidelines. Management information needs may be captured through a manual and/or electronic hospital information system or management information system. The written guidance specifies the frequency of data collection and the person(s) responsible.

5.3 Information management and technology acquisitions

«Hospital Name» defines its need for software and hardware solutions based on current and future information needs. Where electronic medical records are used, relevant national guidance — such as Electronic Health Record standards published by the Ministry of Health and Family Welfare and NABH Digital Health standards — may be referred to. «Hospital Name» ensures it holds the necessary licence for all software in use.

5.4 IT and communication network maintenance

«Hospital Name» implements a maintenance plan for information technology and the communication network, including a specific fire-protection plan covering data server units, telephone exchange units, computers, telephone lines and the nurse-call system. The plan adheres to manufacturer recommendations and includes regular inspections and timely repair of telephone and printer units.

5.5 Contingency planning

«Hospital Name» has a plan to ensure that capture, integration and dissemination of information are not interrupted if the electronic hospital information system experiences downtime. Downtime of the systems is monitored.

5.6 Accuracy of information resources

Information resources at «Hospital Name» — including the website, brochures, newsletters, flyers and educational material — are accurate and meet stakeholder requirements. Accuracy may be ensured through preparation and/or review by subject experts, and feedback and complaints are monitored to verify stakeholder requirements are being met.

5.7 Contribution to external databases

«Hospital Name» sends relevant information at the required frequency and format to concerned authorities as per statutory norms — for example birth and death statistics and notifiable diseases, identified with reference to applicable local, state and national laws, rules, regulations and notifications. «Hospital Name» contributes to databases under relevant national programmes and initiatives, such as Hemovigilance (HVPI), Pharmacovigilance (PVPI) and Materiovigilance (MVPI), where applicable.

5.8 Digital health technology

«Hospital Name» makes efforts to use digital health technology to improve operational efficiency, patient safety and patient experience — for example software for registration, billing, pharmacy, laboratory, imaging, and in-patient and out-patient case records, and the use of artificial intelligence and data analytics where appropriate. Where electronic health systems are used, features that enhance patient safety may be considered, such as computerised physician order entry, clinical decision support, electronic sign-out/hand-off tools, bar-code medication administration, a patient data management system, automated medication dispensing, patient portals, telemedicine, tele-monitoring, electronic incident reporting, and electronic medical records.

6. Governance and responsibility

Role	Responsibility
IT/MIS In-Charge	Owens this policy. Maintains the information-needs assessment, the IT/communication maintenance plan and the contingency plan.
Department Heads / Unit In-Charges	Identify department-level information needs and verify accuracy of resources they produce or use.
Medical Superintendent	Oversees implementation of this policy. Receives escalations under Section 4.
All Staff	Use identified information channels correctly. Report identified risks and rule breaches.

7. Quality monitoring

The IT/MIS In-Charge reviews the information-needs assessment, dissemination records, and IT maintenance and contingency plan currency. Documentary evidence is on file for the CORE objective element and each asterisked objective element (marked CORE and * respectively in the Level column of Section 12, Traceability table). This policy is reviewed annually, or sooner if NABH standards change.

8. Training and staff acknowledgement

All staff covered by this policy are made aware of how information needs are identified and met, and sign the acknowledgement below.

Acknowledgement statement:

"I have read the Policy on Meeting Information Needs of «Hospital Name». I will follow the processes described."

Signature table (one row per staff member):

Staff Name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy is distributed to the IT/MIS department, all department heads/unit in-charges, and is made available to all staff through the hospital intranet and department policy folders.

10. Abbreviations

Abbreviation	Meaning
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
IMS	Information Management System (NABH Hospitals 6th Edition chapter)
OE	Objective Element

11. Traceability to NABH HCO Full Accreditation 6th Edition — IMS.1

This table is an index. It is not how the process is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
IMS.1.a	CORE*	Information needs of stakeholders identified	Section 5.1	IT/MIS In-Charge
IMS.1.b	Commitment *	Identified information needs captured/disseminated	Section 5.2	IT/MIS In-Charge
IMS.1.c	Commitment	IT/technology acquisitions commensurate with needs	Section 5.3	IT/MIS In-Charge
IMS.1.d	Commitment	IT and communication network maintenance plan	Section 5.4	IT/MIS In-Charge
IMS.1.e	Achievement	Contingency plan for continuity of information	Section 5.5	IT/MIS In-Charge
IMS.1.f	Commitment	Information resources accurate, meet stakeholder needs	Section 5.6	IT/MIS In-Charge
IMS.1.g	Commitment	Contributes to external databases per law/regulation	Section 5.7	IT/MIS In-Charge
IMS.1.h	Excellence	Efforts to use digital health technology	Section 5.8	IT/MIS In-Charge

12. Required Records / Evidence Checklist

IMS.1.a — Information needs identified.

- Documented information-needs assessment across patients, visitors, staff, management, external agencies and community.
- Records of the identification mechanism used — feedback forms, patient calls, focus groups, benchmarking.

IMS.1.b — Information needs captured/disseminated.

- Written guidance for capture and/or dissemination of information needs, naming frequency and responsible person(s).
- Dissemination records — website, intranet, booklets, display, signage — and records of information sent to external agencies.

IMS.1.c — IT/technology acquisitions.

- Defined software/hardware needs record based on current and future information needs.
- Software licence records for all software in use.

IMS.1.d — Maintenance plan.

- IT and communication network maintenance plan, including fire-protection plan for servers and telephone exchange units.
- Inspection and repair records for telephone and printer units.

IMS.1.e — Contingency plan.

- Contingency plan document for information capture, integration and dissemination continuity.
- Downtime-monitoring records.

IMS.1.f — Accuracy of information resources.

- Subject-expert preparation/review records for information resources.
- Feedback/complaint monitoring records verifying stakeholder requirements are met.

IMS.1.g — External database contribution.

- Records of information sent to concerned authorities, at the required frequency and format.
- Identified-notifiable-disease list referencing applicable laws/regulations, and vigilance-programme (HVPI/PVPI/MVPI) contribution records where applicable.

IMS.1.h — Digital health technology efforts.

- Record of digital health technology initiatives undertaken (software modules in use, AI/analytics efforts).
- Record of patient-safety-enhancing electronic health system features implemented, where applicable.

13. References

- NABH Guidebook for Accreditation of Hospitals, 6th Edition — Standard IMS.1, "Information needs of the patients, visitors, staff, management and external agencies are met."
- Ministry of Health and Family Welfare, Government of India. (2016). Electronic Health Record (EHR) Standards for India. <https://www.nhp.gov.in/NHPfiles/EHR-Standards-2016-MoHFW.pdf> — applicable where «Hospital Name» uses electronic medical records.
- National Accreditation Board for Hospitals & Healthcare Providers (NABH). (2023). NABH Digital Health Standards for Hospitals (1st ed.). <https://www.koitafoundation.org/src/pdf/nabh-digital-health-standards-for-hospital.pdf> — applicable where «Hospital Name» uses electronic medical records.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed.

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